2.1 Pre-operative Care*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I am afraid I will not wake up after the operation." • Slept poorly the night before surgery **Objective** • HR 104/min, BP 138/88 mmHg • Restless; asks the same questions repeatedly • Hands tremble at rest	**ACTUAL** Excessive anxiety related to anticipated surgery and an unfamiliar environment as evidenced by verbalized fear, restlessness and tachycardia.	• States that he feels calmer before transfer to the operating room. • Heart rate back to baseline within 2 hours.	• Ask him to rate his anxiety and name what frightens him most. • Describe the pre-, intra- and post-operative sequence in plain language. • Leave time for questions and for him to voice his worries. • Let a relative stay until transfer where policy permits. • Coach slow diaphragmatic breathing. • Give the prescribed premedication; record his response.	• Met if he reports less anxiety and HR has returned to baseline. • Persisting distress is reported to the surgeon or anesthetist before transfer.
Subjective • "I do not know what will happen after surgery." • Unsure whether he may eat on the morning of surgery **Objective** • Cannot state his fasting time • Unable to demonstrate deep breathing or an effective cough	**ACTUAL** Inadequate health knowledge related to insufficient exposure to perioperative routines as evidenced by questioning and inaccurate statements about preparation.	• Before surgery, he states his fasting time and pre-operative instructions. • He demonstrates deep breathing, splinted coughing and leg exercises.	• Establish what he already knows and how he prefers to learn. • Explain fasting times and the removal of jewelry, dentures and nail polish. • Teach deep breathing, splinted coughing and leg exercises, then watch a return demonstration. • Introduce the pain scale and outline the post-operative analgesia plan. • Complete the pre-operative checklist and confirm that consent is signed.	• Achieved when the instructions are restated and the exercises performed correctly. • Gaps in learning: repeat the teaching with a family member present.
Subjective • Diabetic for 8 years • Smokes 15 cigarettes a day **Objective** • Blood glucose 190 mg/dL • Booked for elective abdominal surgery • BMI 32 kg/m²	**RISK** Risk for surgical wound infection as evidenced by risk factors: diabetes mellitus, smoking, obesity and planned surgical incision.	• No wound infection over 30 days: incision clean, no erythema or exudate, afebrile.	• Check blood glucose as ordered and report readings above target. • Arrange a preoperative antiseptic shower according to policy. • Do not shave the site; clip hair only if removal is unavoidable. • Confirm that prophylactic antibiotics are given within the ordered window before incision. • Advise him to stop smoking before and after the operation.	• Outcome reached if no sign of infection appears during the admission. • Fever, redness or wound discharge is reported without delay.

* **References for this care plan:** [7], [17], [35], [37], [93], [130]. Full entries are listed in the References section of the CPMN book.